Your Uninsured Decade report

Prepared for Jane - November 14, 2026

Your inputs

Your age	**62**
Spouse / partner age	**60**
State	**Texas**
Approximate net worth	**$500K - $1M**
Current monthly out-of-pocket health spend	**$400**
Hours/week of exercise	**3**
Assumed relative risk reduction	**30%**

Three headline numbers

The Bill: $560,213

Illustrative 10-year staged care cost, adjusted to Texas, in 2034 dollars.

Modeled onset is 70; at 62, that's 8 years out, so this is inflated 4%/yr to 2034 dollars.

The Premium: new money: $33,000

What the basket costs beyond what you already spend, over 10 years.

The full basket is $81,000; you're already spending $48,000, so $33,000 is new.

Years Until Spend-Down: 6 years

At your net worth band, custodial care alone would take about this long to spend you down to Medicaid eligibility.

Based on the 2034-dollar annual private-pay cost, consistent with The Bill above.

The ledger

Line	10-year amount	Notes
Stage 1 - early-stage care	**$45,287**	Mostly in-home support, illustrative
Stage 2 - mid-stage care	**$211,339**	Heavier in-home / adult-day support
Stage 3 - late-stage custodial care	**$303,587**	Memory care / nursing home, state-adjusted
The Bill (total)	**$560,213**	Sum of stages 1-3, in onset-year dollars
The Premium - full basket (10 years)	**$81,000**	4M protocol basket, $675/mo ($81,000 over 10 yrs)
Already spending (10 years)	**$48,000**	Your reported current monthly health spend x 120
New money (10 years)	**$33,000**	The actual new spend the basket requires
Hours to add per month	**0**	You're already paying the hours; the ledger below is about the dollars.
Expected savings, dementia-cost avoidance only [assumption, not a guarantee]	**$111,258**	Compared against the full basket - the honest premium

Expected value, stated honestly: for the two of you

This is the part most calculators fudge. We won't. Nothing lowers your risk to zero, and nothing "prevents" dementia - the honest claim is that a sustained, structured lifestyle protocol can lower the odds. Here's what that's worth in dollars, using an assumption you control:

Lifetime risk of any dementia after age 55 is **42%** (or, for a couple, about a **66%** chance at least one partner is affected) - per the largest long-running U.S. cohort study to date (Fang, Coresh et al., Nature Medicine, 13 Jan 2025, ARIC cohort).

Assumed relative risk reduction: **30%**

45% = Lancet Commission 2024 ceiling if all 14 modifiable risk factors are addressed (a population figure, not a personal guarantee). This is an assumption, not a guarantee - see disclaimer below.

Unmitigated expected cost (probability-weighted): **$370,861**. With a 30% assumed risk reduction: **$259,603**. Expected savings: **$111,258** against a $81,000 ten-year premium - the full basket, the honest comparison. **At these inputs the expected savings exceed the premium.** Read that carefully: the premium is money you will certainly spend; the savings are an expectation spread across odds you cannot see in advance. Nobody should buy this on the arithmetic alone.

Read plainly: whichever way your number lands, dementia-cost avoidance is a single-disease bet, and this is never a single-disease decision. A couple at the default 30% assumed risk reduction shows expected savings of roughly $80,485 against a $81,000 ten-year premium; at the 45% Lancet ceiling, roughly $120,728. The honest case for the spend is that the ledger really tips once the same modifiable factors' effect on diabetes, cardiovascular disease, and stroke - plus the wide right tail past the $405,262 average cost, plus quality-of-life and caregiver-hour value - are counted alongside dementia. None of that is captured in the math above. Insurance is a ruin-avoidance trade, not an ROI calculation - the same trade you already make on your house. You don't insure your house because you expect it to burn down; you insure it because you can't afford it if it does.

What the ledger can't count. The same premium that may lower the odds of the bill pays a dividend now, whether or not a claim ever comes: sleep, strength, energy, a retirement decade spent on your feet. That dividend is worth more than any number above, and it is the reason the people who do this keep doing it. What it protects was never really the money. Don't lose your identity and your dignity while you still have a choice.

A policy funds the risk; it can't lower it - that part is on you. Take this printout to the advisor who sent you here, and review your plan, policies, powers of attorney, and health-care directives with them - that step is necessary, not optional.

One more caveat: the 42% figure counts all dementia, including mild cases diagnosed late in life - so treat this as a range to reason with, not a promise about your own outcome.

Review this report with your financial advisor, estate attorney, or CPA. They protect the money. This protects the person the money is for. You need both.

Don't lose your identity and your dignity while you still have a choice.

Medical Disclaimer: My4MLife provides access to telehealth services and prescription medications through licensed healthcare providers and pharmacies. Compounded medications are not FDA-approved. Individual results may vary. GLP-1 medications are prescription-only and require a physician evaluation. This website does not provide medical advice. Always consult with a qualified healthcare provider regarding your health. Testimonials reflect individual experiences and are not guarantees of results. Images may use models for privacy.

These figures are illustrative estimates based on published third-party cost-of-care data (Alzheimer's Association, CareScout/Genworth, USC Schaeffer Center) and a hypothetical relative-risk-reduction assumption. This is not financial, tax, insurance, or legal advice, and no protocol guarantees a reduction in your personal risk. Consult your own financial advisor and physician.